advancement of pharmaceutical sciences and public health. The NBFC's integrated approach is pivotal as we face challenges of urbanisation and environmental degradation, focusing on enhancing biodiversity, fostering green jobs, and improving global health by integrating biodiversity and human wellbeing indices. Aligned with the European Resilience Plan and focusing on the youth, the NBFC is committed to ensuring future generations live in harmony with nature and achieve the Sustainable Development Goals. We trust that this Correspondence will underscore the importance of biodiversity in safeguarding not only our planet's health, but also the health and prosperity of its inhabitants.

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See **Online** for appendix

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Immunotherapy in frail non-small-cell lung cancer patients

We read the study published by Siow Ming Lee and colleagues[1] with great interest. The authors concluded that atezolizumab monotherapy was associated with better outcomes for patients deemed ineligible for platinum-based chemotherapy. However, the study raised two substantial concerns regarding the heterogeneity of the study population and the single-agent treatment of the control group.

First, the trial combined frail and very frail patients with an Eastern Cooperative Oncology Group performance status (ECOG PS) 2 and 3 with a substantial number of fit (ECOG PS 0–1) older patients. These are three very distinctive groups who respond differently to treatment.[2] There was no difference between immunotherapy or chemotherapy in ECOG 2 (hazard ratio [HR] 0·86, 95% CI 0·67–1·10). The paper even stated that the median overall survival of ECOG PS 2 patients treated with chemotherapy was better than those treated with immunotherapy (9·7 *vs* 10·4 months). The question here is whether the benefit of the total group might be driven by the fit older patient ECOG PS 0–1 group.

Second, IPSOS investigators possibly deemed platinum-based therapy unsuitable for patients primarily by their performance score. A 2023 Cochrane review, with IPSOS data included, showed that patients with ECOG PS 2 should be treated with platinum doublet therapy first, not non-platinum monotherapy (HR 0·67 [0·57–0·78]), contrary to its use in this trial.[3] Furthermore, the observed crossing of the survival curve during the first months could be attributed to withholding chemotherapy in a substantial part of the atezolizumab group. Consequently, the findings from this study for patients with ECOG PS 2, should be interpreted with caution.

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Comprehensive inclusion: demographics of clinical trials

We read with great interest Siow Ming Lee and colleagues'[1] study on the efficacy and safety of atezolizumab in patients with non-small-cell lung cancer. The research commendably addresses a broader demographic of lung cancer patients who are less commonly included in clinical trials compared with patients who have solid organ tumours.[2] But an unmistakable concern arises: the grave under-representation of Black patients.

In a cohort spanning 91 clinical sites globally, this trial had a mere 1% Black representation equating to only three Black participants. This glaring disparity prompts questions about the effectiveness of inclusivity efforts. Historical and ongoing clinical research shows that factors such as population stratification, genetic diversity, and sociocultural differences can profoundly influence drug efficacy and safety outcomes.[3] Overlooking or marginalising a race or ethnicity can compromise the applicability and universality of the findings.

Clinical research has the esteemed role of paving the way for medical advancements. With this power, however, comes a substantial responsibility—the duty to confront and rectify systemic biases. The oversight in this study reflects broader, structural obstacles that plague clinical trials.[4] As stated before, a solution meant for all but available only to a select few is not an ideal remedy. Instead, it is a harbinger of further disparities.

The global medical community should advocate fervently for equal representation.[5] Although studies might highlight promising drugs, it is paramount to gauge their efficacy across all racial and ethnic groups adequately. True scientific triumph lies not in the breadth of discovery but in its depth of equitability.

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Totality of evidence refutes neoplasm risk with fezolinetant

We read with interest the Correspondence from Jonathan Douxfils and colleagues regarding an increased risk of neoplasm with the neurokinin 3 receptor antagonist fezolinetant.[1] However, important considerations warrant clarification.

The numeric imbalance identified in SKYLIGHT 4 (see appendix 4 in the full-length study paper)[2] prompted an in-depth analysis of all benign, malignant, and unspecified neoplasms from phase 2 and 3 fezolinetant studies. This analysis was combined with a comprehensive assessment of key characteristics of carcinogens,[3] an evaluation of fezolinetant structural properties and non-clinical data, and an epidemiological literature review. Given a short latency period to diagnosis, heterogeneity of tumour type, previous neoplasm or risk factor history, and presence of alternative baseline causes, a drug effect is not supported. Considering pre-existing malignancies, the US Food and Drug Administration concluded that "this brings the incidence rate of malignancy TEAEs [treatment-emergent adverse events] within the normal background rate of cancer".[4] Furthermore, no plausible mechanism exists for neurokinin 3 receptor antagonism in neoplasms. Importantly, although tachykinin agonism could be involved in neoplastic development, antagonism has been considered as a potential antineoplastic target.

Douxfils and colleagues carried out a meta-analysis with Peto odds ratios to assess the risk of neoplasms with fezolinetant.[1] Studies have shown that the Peto methodology is not appropriate for investigating rare events and is not an advocated default approach for meta-analyses.[5,6] Given different study designs and exposure durations, combining the SKYLIGHT studies into a meta-analysis is also not recommended and could lead to flawed interpretations.

In conclusion, the totality of current non-clinical and clinical data from the fezolinetant development programme does not support a signal for an increased risk of neoplasms.

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